# Temozolomide with short-course radiotherapy, then adjuvant treatment

Temozolomide chemotherapy to help control glioma, with radiotherapy in the initial phase of this plan.

**Adults older than 65 with newly diagnosed glioblastoma who are not suitable for the standard radiotherapy regimen with temozolomide, under NCCP 00461.** · NCCP 00461 v3 · Document revised 5 November 2026

**Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Adults older than 65 with newly diagnosed glioblastoma who are not suitable for the standard radiotherapy regimen with temozolomide, under NCCP 00461. This card records adjuvant cycles separately from concurrent radiotherapy.

## My overall treatment

Daily temozolomide with 3-week radiotherapy (RT) → 4-week break → D1–5 every 28 days, up to 6 cycles.

**28** day view

Treatment timing in this window

**Temozolomide once daily** — Day 1–5

**Up to six adjuvant cycles**

Doses, starts and stopping are confirmed by the team; a card stamp is a record, not approval.

## A typical 28-day cycle

| Day 1–5 | Day 6–21 | Day 22–28 |
|---|---|---|
| Five daily capsule doses | No temozolomide; recovery continues | Blood count and next-cycle decision |

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

## My tests and checks

| When | What is checked | Timing / reason |
|---|---|---|
| **Before** | **Bloods / infection screen**<br>Required by regimen | Counts, kidney/liver, glucose and hepatitis B screen. |
| **RT + recovery** | **Weekly counts, weeks 1–7**<br>Required by regimen | Kidney/liver tests 1 and 4 weeks after RT. |
| **Adjuvant / after** | **Cycle safety / recovery**<br>NCCP + EMA / individual scan plan | Day 1 bloods; Day 22 count and after-cycle liver tests (EMA). magnetic resonance imaging (MRI) on your plan. |

## Between treatments

- The 28-day calendar is for adjuvant treatment. The earlier daily RT phase has different doses and tests.
- Keep Pneumocystis jirovecii pneumonia (PJP) prevention and other support medicines as prescribed; do not stop them automatically during the break.

Use the current pharmacy label and anti-sickness plan. Swallow capsules whole. Never double or replace a vomited dose.

**When to get help**

**Call 112** / **999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**



Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Brain / energy

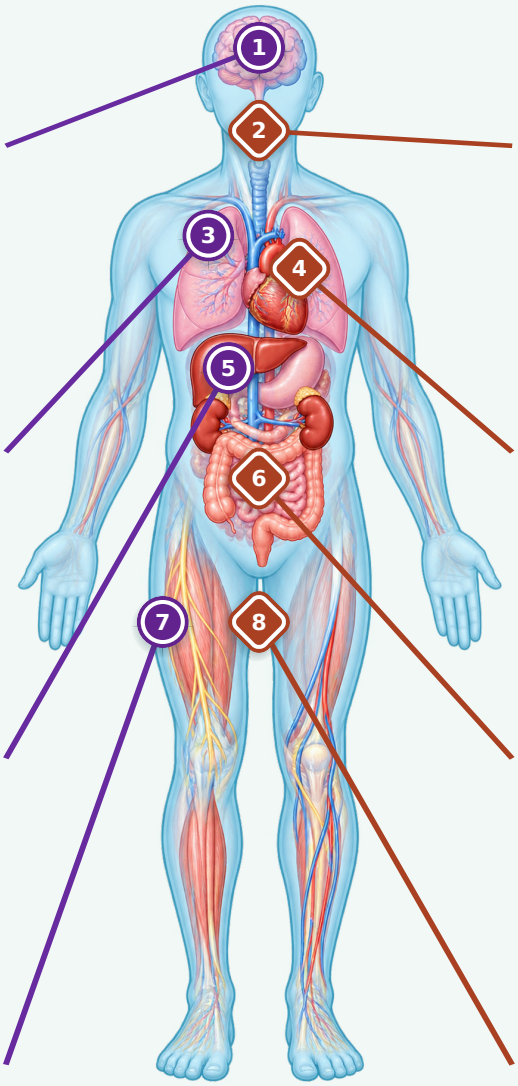
**Tumour / treatment**

Fatigue or headache may occur. New seizures, sudden weakness, speech difficulty or loss of consciousness need emergency help.

### 2 Mouth / eating

**Temozolomide**

Report mouth soreness, ulcers or reduced intake. Keep gentle mouth care and ask for help before eating or drinking becomes difficult.




### 3 Blood / infection

**Temozolomide**

Counts can fall after capsule days finish. Fever, sudden illness, bruising or bleeding needs urgent advice; keep all blood tests.

### 4 Lungs / PJP

**Temozolomide**

New cough, fever or breathlessness needs prompt review. Follow the prescribed PJP-prevention plan during chemoradiotherapy.

### 5 Liver

**Temozolomide**

Liver injury may appear during or after treatment. Attend liver tests and report yellow skin or eyes, dark urine or unusual itching.

### 6 Nausea / bowels

**Temozolomide / support medicines**

Take anti-sickness medicine as prescribed. Call for repeated vomiting, inability to drink, severe constipation or abdominal pain.

### 7 Skin / scalp

**Radiotherapy / temozolomide**

Follow scalp care during RT; report new rash, blisters or peeling skin. Hair loss and skin irritation depend on the treatment received.

### 8 Fertility / pregnancy

**Temozolomide**

Discuss contraception and fertility before starting. Do not breastfeed during treatment. Ask for the written medicine-specific precautions.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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# Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

**FRONT / CONTACTS**

## My Treatment Card

SACTCheck

Name: _____

**Temozolomide + short RT**

Daily temozolomide with 3-week RT → 4-week break → D1–5 every 28 days, up to 6 cycles.

Phase: _____ Planned cycles: 6

**Daytime (EXAMPLE)** 087 12345678

**Out of hours (EXAMPLE)** 087 87654321

**Unwell or fever? Call oncology now.**
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

**BACK / TREATMENT RECORD**

## My cycle record

Team stamps after each completed cycle.

| C1 | C2 | C3 |
|---|---|---|
| Date / initials | Date / initials | Date / initials |
| C4 | C5 | C6 |
| Date / initials | Date / initials | Date / initials |

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

**FOLD centre — do not cut**

## Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

## Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

### iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

### Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

## Your local contacts and resources

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.


Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.